

FLORA WELLNESS

Complete Guide Series | Volume 01

The Hormone Reset Protocol

Your Complete 8-Week Guide to Naturally
Rebalancing Your Hormones

Science-backed strategies for estrogen, progesterone, cortisol & beyond

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SECTION 1: UNDERSTANDING YOUR HORMONAL LANDSCAPE

What Are Hormones and How Do They Work?

Hormones are chemical messengers produced by glands throughout the endocrine system. They travel through your bloodstream and attach to receptor sites on cells, triggering biological responses that regulate virtually every function in your body — from your mood and metabolism to your sleep cycles and reproductive health. What makes hormones so powerful is their potency: even changes measured in picograms per milliliter (one trillionth of a gram) can produce dramatic shifts in how you feel and function.

Your body produces over 50 distinct hormones. While they each have specific individual roles, they work in intricate feedback loops and cascades, meaning a disturbance in one hormone rarely stays isolated — it ripples outward, affecting the entire system. This is why hormone imbalances often present as a confusing constellation of seemingly unrelated symptoms: fatigue, weight gain, anxiety, hair loss, irregular periods, and brain fog, all stemming from one root disruption.

Understanding how your hormones function — and how modern life systematically disrupts them — is the foundation of the Hormone Reset Protocol. When you address the root causes rather than chasing individual symptoms, you create conditions where your body can recalibrate itself naturally.

The Endocrine System: Your Body's Command Centre

The endocrine system is a network of glands and organs that produce, store, and release hormones. At the top of the hierarchy sits the hypothalamus — a small region in your brain that acts as the master regulator, monitoring the body's internal environment and sending chemical signals to the pituitary gland. The pituitary, often called the 'master gland,' then releases its own hormones that direct other glands: the thyroid, adrenal glands, ovaries, and pancreas.

This hierarchy operates through feedback loops. When the ovaries produce enough estrogen, they send a signal back to the hypothalamus and pituitary to reduce the stimulating signals. When cortisol drops, the hypothalamus increases CRH, prompting more cortisol production. These feedback loops maintain homeostasis — until they're overwhelmed by chronic stress, toxins, nutrient deficiencies, or inflammation.

- Hypothalamus: master regulator, monitors internal environment, releases CRH, GnRH, TRH

- Pituitary gland: releases FSH, LH, TSH, ACTH, growth hormone, prolactin
- Thyroid gland: produces T3 and T4 for metabolism, temperature, and energy
- Adrenal glands: produce cortisol, DHEA, aldosterone, and adrenaline
- Ovaries: produce estrogen, progesterone, and testosterone
- Pancreas: produces insulin and glucagon for blood sugar regulation
- Pineal gland: produces melatonin to regulate the sleep-wake cycle

Key Hormones for Women: Functions, Imbalance Signs & Optimal Ranges

Hormone	Key Functions	Signs Too Low	Signs Too High	Optimal Range
Estradiol (E2)	Bone density, mood, skin elasticity, vaginal health, cognitive function	Hot flashes, dry skin, low mood, poor memory, UTIs	Heavy periods, breast tenderness, bloating, weight gain, fibroids	100-400 pg/mL (phase-dependent)
Progesterone	Calming, sleep quality, uterine lining, counterbalances estrogen	Anxiety, PMS, insomnia, spotting, irregular cycles, miscarriage	Drowsiness, weight gain (rare)	10-25 ng/mL (mid-luteal)
Testosterone	Libido, muscle mass, motivation, confidence, bone density	Low sex drive, muscle wasting, depression, fatigue, poor focus	Acne, oily skin, excess hair, aggression, PCOS	15-70 ng/dL (total)
Cortisol	Stress response, blood sugar, anti-inflammatory, waking energy	Fatigue, low blood pressure, poor immunity, salt cravings	Belly fat, anxiety, insomnia, blood sugar swings, muscle breakdown	10-20 mcg/dL (8am)

Hormone	Key Functions	Signs Too Low	Signs Too High	Optimal Range
Insulin	Glucose uptake, energy storage, cell signalling	Hypoglycemia, shakiness, irritability, confusion	Weight gain, PCOS, type 2 diabetes, inflammation, brain fog	2-8 mIU/L (fasting)
Free T3 (thyroid)	Metabolic rate, temperature regulation, heart rate, mood	Fatigue, weight gain, hair loss, cold hands/feet, constipation	Heart palpitations, anxiety, insomnia, weight loss, sweating	3.2-4.2 pg/mL
DHEA-S	Adrenal reserve, testosterone precursor, immune modulation, vitality	Low energy, poor immunity, loss of muscle, low libido	Acne, oily skin, excess facial hair	65-380 mcg/dL
Melatonin	Sleep initiation, circadian rhythm, antioxidant, ovarian support	Insomnia, poor sleep quality, irregular cycles, low antioxidant	Excessive sleepiness, grogginess	Peak >80 pg/mL (2-3am)

The Hormone Cascade: How One Imbalance Triggers Many

Hormone imbalances almost never occur in isolation. The most common cascade begins with chronic stress elevating cortisol. High cortisol triggers the 'pregnenolone steal' — diverting the precursor hormone pregnenolone away from sex hormone production (progesterone, estrogen, DHEA, testosterone) toward more cortisol synthesis. This simultaneously drops progesterone (causing estrogen dominance), depletes DHEA (accelerating aging), and reduces testosterone (crushing libido and muscle mass).

Elevated cortisol also suppresses thyroid function by blocking the conversion of inactive T4 to active T3, creates insulin resistance that drives weight gain and PCOS, and suppresses melatonin so sleep suffers — which further elevates cortisol the next day. One root cause creates a self-perpetuating hormonal

nightmare.

The second common cascade starts with insulin resistance: high insulin drives androgen production in the ovaries, causing PCOS. Excess androgens block ovulation, causing low progesterone and irregular cycles. Low progesterone allows estrogen dominance, increasing cancer and fibroid risk. All from blood sugar dysregulation.

The Key Principle

Address the root of your hormone cascade — not individual symptoms. Fix the cortisol, fix the blood sugar, fix the gut — and the downstream hormones often normalize on their own. This is why the Hormone Reset Protocol works from the foundation up.

SECTION 2: THE 5 HORMONE TYPES

Identifying your primary hormone type lets you stop guessing and start targeting. While most women have overlapping elements, one or two types dominate. Use the symptom patterns below along with your lab results to identify your type, then follow the targeted protocol.

TYPE 1: ESTROGEN DOMINANT

Signs & Symptoms: Heavy or clotted periods, breast tenderness and fibrocystic breasts, bloating and water retention, PMS mood swings and rage, weight gain on hips and thighs, headaches before period, endometriosis, fibroids, low libido, fatigue

Root Causes: Xenoestrogen exposure (plastics, pesticides, personal care products), liver congestion slowing estrogen clearance, gut dysbiosis increasing estrogen reabsorption, low progesterone from stress, obesity (fat cells produce estrogen), alcohol consumption

Targeted Protocol: Daily cruciferous vegetables + DIM supplementation, liver detox protocol, calcium D-glucarate, gut microbiome repair, Vitex for progesterone support, reduce xenoestrogen exposure aggressively

TYPE 2: PROGESTERONE DEFICIENT

Signs & Symptoms: Anxiety and nervousness particularly before period, irregular cycles or cycles shortening over time, spotting or brown discharge before period starts, severe PMS in luteal phase, insomnia and restless sleep, recurrent miscarriages, low libido, short luteal phase

Root Causes: Chronic stress causing cortisol steal from progesterone, anovulatory cycles (no ovulation = no progesterone), perimenopause transition, xenoestrogen exposure creating relative deficiency, insufficient cholesterol intake for hormone production

Targeted Protocol: Stress reduction as first priority (no supplement replaces this), Vitex 400mg every morning, magnesium glycinate 400mg nightly, B6 50-100mg, seed cycling, consider bioidentical progesterone cream with practitioner guidance

TYPE 3: CORTISOL DYSREGULATED

Signs & Symptoms: Morning fatigue despite full night's sleep, wired but tired feeling, 2-4pm energy crash, weight gain specifically in belly/face, heightened anxiety and racing thoughts, disrupted sleep (waking 2-4am), frequent illness, salt and sugar cravings, brain fog

Root Causes: Chronic psychological stress (work, relationships, finances), blood sugar roller coasters feeding cortisol spikes, over-exercise and under-recovery, chronic under-eating, gut infections taxing the immune system and adrenals, unresolved emotional trauma

Targeted Protocol: Adrenal nutrition protocol (protein + fat at every meal), adaptogens (ashwagandha, rhodiola, holy basil), 4-point cortisol testing, sleep optimization, nervous system regulation practices, eliminate HIIT during recovery phase

TYPE 4: THYROID SLUGGISH

Signs & Symptoms: Persistent fatigue that doesn't improve with sleep, unexplained weight gain or inability to lose weight, hair loss (diffuse, including eyebrow outer thirds), cold hands and feet, constipation, brain fog and poor memory, dry skin and brittle nails, low mood and depression

Root Causes: Hashimoto's autoimmune thyroiditis (most common cause), selenium and iodine deficiency, gluten triggering autoimmune attack on thyroid, gut permeability, chronic stress suppressing T4 to T3 conversion, environmental toxins (fluoride, bromide, chlorine)

Targeted Protocol: DUTCH test or comprehensive thyroid panel including antibodies, selenium 200mcg, iodine from food sources, strict gluten elimination trial (90 days), gut healing protocol, reduce fluoride exposure, work with functional medicine practitioner on dosing

TYPE 5: INSULIN RESISTANT

Signs & Symptoms: Weight gain especially around the belly despite eating well, intense sugar and carbohydrate cravings, energy crashes 1-2 hours after meals, skin tags, dark patches in skin folds (acanthosis nigricans), PCOS symptoms, blood sugar over 85 fasting, irregular periods

Root Causes: High refined carbohydrate and sugar diet, sedentary lifestyle, chronic stress elevating cortisol and blood sugar, poor sleep increasing insulin resistance, gut dysbiosis, genetic predisposition, polycystic ovary syndrome

Targeted Protocol: Low glycemic whole foods diet, protein and fat at every meal, post-meal 10-minute walks, berberine 500mg with meals (as effective as metformin in studies), inositol 4g daily for PCOS, chromium 400mcg, time-restricted eating window

SECTION 3: HORMONE RESET NUTRITION PROTOCOL

The Foundational Eating Principles

Food is the most powerful hormone-modulating tool you have, and you use it multiple times every day. Every meal either moves you toward hormone balance or away from it. The Hormone Reset nutrition approach isn't a diet — it's a framework for using food as targeted medicine for your endocrine system.

The four non-negotiable principles: First, stabilize blood sugar at every meal (protein + fat + fiber at minimum). Second, eat enough — chronic under-eating spikes cortisol and crashes progesterone. Third, eliminate the hormone disruptors (refined sugar, alcohol, non-organic soy, conventional dairy). Fourth, actively add hormone-supporting foods daily.

Estrogen-Balancing Foods: The Full Guide

Cruciferous vegetables are the cornerstone of estrogen balance. Broccoli, cauliflower, Brussels sprouts, kale, cabbage, bok choy, arugula, and watercress all contain glucosinolates that convert to indole-3-carbinol (I3C) in the gut, which then converts to DIM (diindylmethane). DIM acts as a selective estrogen receptor modulator, shifting estrogen metabolism away from the harmful 16-alpha-hydroxyestrone toward the protective 2-hydroxyestrone pathway. Target 1-2 cups daily, lightly steamed (4 minutes) to preserve myrosinase enzyme activity.

Flaxseeds provide lignans — plant compounds that compete with estrogen for receptor binding, reducing the overall estrogenic load. Two tablespoons of freshly ground flaxseed daily has been shown in multiple studies to improve estrogen metabolism, reduce breast density, and lower cancer risk markers. Always grind fresh (pre-ground oxidizes quickly) and store in the fridge. Add to smoothies, yogurt, oatmeal, or baked goods.

Fiber is critical for estrogen elimination. Your liver packages used estrogen for elimination via bile into the bowel. If you're constipated or eating low fiber, this estrogen gets deconjugated by gut bacteria and reabsorbed — a major driver of estrogen dominance. Target 30-35g fiber daily from vegetables, legumes, berries, and seeds. Daily bowel movements are non-negotiable for hormone balance.

- **Broccoli sprouts:** Highest concentration of sulforaphane; 3-day-old sprouts have 50x more than mature broccoli
- **Pomegranate:** Contains ellagitannins that inhibit aromatase (enzyme converting testosterone to estrogen)
- **Flaxseed (2 tbsp ground):** Lignans shift estrogen metabolism to protective pathways
- **Turmeric:** Curcumin reduces aromatase activity and decreases estrogenic signalling
- **Green tea (EGCG):** Inhibits aromatase, reduces estrogen receptor sensitivity
- **Rosemary:** Carnosol inhibits aromatase and enhances Phase 2 liver detox
- **Citrus zest:** D-limonene activates liver detox enzymes for estrogen clearance
- **Beets:** Betaine supports liver methylation for estrogen metabolism

7-Day Hormone Reset Meal Plan

Day	Breakfast	Lunch	Dinner	Snack
Mon	Seed cycling smoothie: flaxseed, blueberries, spinach, almond butter, coconut milk	Cruciferous bowl: shredded kale, roasted broccoli, chickpeas, tahini-lemon dressing	Wild salmon, steamed Brussels sprouts, roasted sweet potato	Apple + almond butter
Tue	Overnight oats: flaxseed, chia, berries, cinnamon, walnuts	Sardine & arugula salad, avocado, olive oil, lemon	Grass-fed beef stir-fry, bok choy, broccoli, cauliflower rice	Handful mixed nuts + herbal tea
Wed	Green smoothie: spinach, avocado, cucumber, ginger, hemp seeds, lemon	Lentil soup with kale, carrots, turmeric, garlic	Organic chicken thigh, roasted cauliflower, quinoa pilaf	Carrot sticks + hummus

Day	Breakfast	Lunch	Dinner	Snack
Thu	2 eggs, sautéed spinach, half avocado, microgreens	Wild tuna salad: arugula, capers, olives, olive oil, lemon	Lamb & veggie curry, coconut milk base, cauliflower rice	85% dark chocolate + peppermint tea
Fri	Chia pudding: coconut milk, vanilla, berries, pumpkin seeds	Roasted veggie & quinoa bowl, tahini, fresh herbs	White fish en papillote with asparagus, cherry tomatoes, herbs	Rice cake + almond butter + banana
Sat	Almond flour pancakes with flaxseed, fresh berries, maple drizzle	Grain bowl: farro, roasted veggies, soft egg, miso-ginger dressing	Grass-fed burger, lettuce wrap, sweet potato fries, avocado	Trail mix: nuts, seeds, coconut flakes
Sun	Smoothie bowl: acai, banana, topped with granola, hemp seeds, berries	Bone broth soup: kale, lentils, ginger, turmeric, vegetables	Roasted chicken, root vegetables, herb gravy, steamed greens	Celery & almond butter + herbal tea

Complete Supplement Protocol

Supplement	Daily Dose	Timing	Key Hormone Benefit	Notes
Magnesium Glycinate	300-400mg	Evening before bed	Progesterone support, cortisol reduction, PMS, sleep	Most bioavailable form; start low and increase
DIM	100-200mg	With largest meal	Shifts estrogen to safer metabolites, liver support	Take with fat for absorption; use if estrogen dominant

Supplement	Daily Dose	Timing	Key Hormone Benefit	Notes
Vitex / Chasteberry	400-600mg	Morning, empty stomach	Progesterone stimulation, cycle regulation, PMS	Requires 3+ months; do not use with hormonal BC
Vitamin B6 (P5P)	50-100mg	With meals	Progesterone synthesis, estrogen metabolism, mood	P5P form is active — much better absorbed than B6
Zinc	15-30mg	With dinner	Progesterone, testosterone, thyroid, immunity	Take with food to avoid nausea; pairs well with copper
Omega-3 (EPA+DHA)	2-3g combined	With food (any meal)	Inflammation, prostaglandins, mood, hormone signalling	Triglyceride form better absorbed; keep refrigerated
Vitamin D3 + K2	2000-5000 IU D3	With fat-containing meal	Hormone synthesis, insulin sensitivity, thyroid	Add K2 (100-200mcg MK-7) to prevent calcification
Calcium D-Glucarate	500mg	With meals	Inhibits beta-glucuronidase, clears excess estrogen	Critical if constipated; take with meals
N-Acetyl Cysteine	600mg	Away from meals	Liver glutathione, PCOS, anti-inflammatory	Morning or afternoon on empty stomach
Ashwagandha KSM-66	300mg x2	Morning + evening	Cortisol reduction, thyroid, testosterone	Cycle: 3 months on, 1 month off

Supplement	Daily Dose	Timing	Key Hormone Benefit	Notes
Inositol (myo + D-chiro)	4g total (40:1 ratio)	With meals	PCOS, insulin sensitivity, ovulation support	Especially important for PCOS Type 5
Selenium	200mcg	Morning	Thyroid T4→T3 conversion, antioxidant, anti-Hashimoto's	Brazil nuts = 70-90mcg each; don't exceed 400mcg/day

SECTION 4: THE LIVER & GUT DETOX PROTOCOL

Understanding Liver Detox Phases 1 & 2

Your liver is the primary organ responsible for metabolizing and clearing hormones. It processes estrogen through a two-phase detoxification system. Phase 1 uses a family of enzymes called cytochrome P450 to convert active estrogen into intermediate metabolites. Phase 2 then attaches molecules (glucuronide, sulfate, glutathione) to these intermediates to make them water-soluble for elimination.

Problems arise when Phase 1 is overactive and Phase 2 can't keep up — creating a backlog of reactive estrogen intermediates that damage DNA and increase breast cancer risk. Or when Phase 2 is sluggish due to nutrient deficiencies (B vitamins, amino acids, magnesium), and estrogen isn't cleared efficiently. Supporting both phases equally is the goal.

Detox Phase	What Happens	Nutrients Required	Foods That Support	Impaired By
Phase 1 (Hydroxylation)	P450 enzymes oxidize estrogen into intermediate forms	B2, B3, B6, B12, folic acid, glutathione, magnesium, phospholipids	Cruciferous veg, berries, citrus, rosemary, green tea	Alcohol, grapefruit, some medications, high toxin load
Phase 2 (Conjugation)	Intermediates are packaged for elimination via bile or urine	Glycine, taurine, glutamine, cysteine, B5, B12, folate, sulfur	Eggs, garlic, onions, broccoli sprouts, cruciferous veg, beets	Low protein diet, amino acid deficiencies, oxidative stress

Detox Phase	What Happens	Nutrients Required	Foods That Support	Impaired By
Bile Flow	Conjugated hormones exit in bile into intestines	Adequate bile, choline, taurine	Beets, artichokes, bitter greens, dandelion, radishes	Constipation — estrogen gets reabsorbed; gallbladder disease

The Estrobolome: Gut Bacteria & Estrogen Recycling

The estrobolome is a collection of gut bacteria that produce an enzyme called beta-glucuronidase. This enzyme cleaves the glucuronide tag from conjugated estrogen in the gut, freeing it to be reabsorbed back into circulation instead of eliminated. When gut dysbiosis is present — an overgrowth of beta-glucuronidase producing bacteria — significantly more estrogen recirculates, contributing powerfully to estrogen dominance.

Studies have shown that women with higher beta-glucuronidase activity have measurably higher circulating estrogen levels, greater risk of hormone-sensitive cancers, and more severe PMS. Addressing gut health is therefore not optional for hormone balance — it's foundational. Calcium D-glucarate supplementation directly inhibits beta-glucuronidase activity, providing immediate support while you rebuild the microbiome.

- Take calcium D-glucarate 500mg with meals to inhibit beta-glucuronidase immediately
- Add fermented foods daily: kefir, kimchi, sauerkraut, kombucha, miso
- Take a high-quality probiotic (50 billion CFU, multi-strain) with dinner
- Eat 30+ different plant foods weekly to maximize microbiome diversity
- Add prebiotic fiber: garlic, onions, asparagus, Jerusalem artichoke, green banana
- Address constipation urgently — target 1-2 formed bowel movements daily
- Consider a 4R gut protocol if dysbiosis, SIBO, or leaky gut is suspected

SECTION 5: STRESS, CORTISOL & ADRENAL SUPPORT

The Pregnenolone Steal: How Stress Hijacks Your Hormones

Pregnenolone is the 'mother hormone' — the raw material from which your body makes all steroid hormones including progesterone, estrogen, testosterone, cortisol, and DHEA. Under normal conditions, pregnenolone is distributed across all these hormones according to your body's needs. Under chronic stress, the adrenal glands demand an increasing share of pregnenolone to produce cortisol — literally stealing it away from sex hormone production.

The result: progesterone drops (causing estrogen dominance, irregular cycles, anxiety, and insomnia), DHEA declines (accelerating aging and reducing vitality), and testosterone falls (reducing libido, muscle mass, and drive). All from one root cause: too much cortisol demand. This is why stress management isn't a 'nice to have' in hormone health — it's the most critical intervention for many women.

The Adaptogen Protocol

Adaptogen	Best For	Dose & Form	Research Evidence	Cycle Protocol
Ashwagandha KSM-66	High cortisol, sleep, thyroid, testosterone	300mg 2x daily (standardized extract)	Reduces cortisol 27%, improves thyroid T3, boosts testosterone	3 months on, 1 month off
Rhodiola Rosea	Mental burnout, cognitive fatigue, low mood, physical performance	200-400mg morning (3% rosavins)	Reduces cortisol, improves work performance, anti-fatigue in 3 days	Cycle: 6 weeks on, 2 weeks off
Holy Basil (Tulsi)	Anxiety, blood sugar, gentle daily adrenal support	600mg or 2 cups tea daily	Reduces cortisol and blood glucose, anxiolytic effect	Continuous; very gentle

Adaptogen	Best For	Dose & Form	Research Evidence	Cycle Protocol
Eleuthero	Adrenal burnout recovery, low energy, poor immunity	400mg 2x daily	HPA axis modulation, reduces fatigue, improves endurance	3 months on, 1 month off
Maca	Hormonal balance, libido, menopause symptoms, energy	1.5-3g (gelatinized form)	Supports estrogen/testosterone balance, reduces menopause symptoms	Continuous; food-based
Schisandra	Liver + adrenal + cognitive support, multi-action	500-1000mg or as tea	Reduces cortisol, liver cytoprotective, improves cognitive performance	Continuous use safe
Reishi Mushroom	Immune modulation, sleep quality, cortisol, autoimmunity	1.5-9g or 500-2000mg extract	Reduces cortisol, improves deep sleep, immunomodulatory	Continuous; very safe

SECTION 6: THE 8-WEEK HORMONE RESET PLAN

WEEK 1: ELIMINATE & STABILIZE

- ✓ Remove the 5 major disruptors: alcohol, caffeine, refined sugar, gluten (trial), conventional dairy
- ✓ Eat protein + fat + fiber at every meal — no naked carbohydrates ever
- ✓ Start magnesium glycinate 300mg at bedtime, omega-3 2g with dinner
- ✓ Establish fixed sleep/wake times (within 30 min, including weekends)
- ✓ 20-minute morning sunlight walk within 1 hour of waking
- ✓ No screens 60 min before bed; use blue-light glasses in evenings
- ✓ Rate your top 10 symptoms 1-10 in your journal as your baseline
- ✓ Add 1 cup cruciferous vegetables daily (broccoli, kale, Brussels sprouts)

WEEK 2: BLOOD SUGAR MASTERY

- ✓ Follow the Hormone Reset Plate: $\frac{1}{2}$ veg, $\frac{1}{4}$ protein, $\frac{1}{4}$ complex carb, fat always present
- ✓ Take a 10-minute walk after every main meal for insulin sensitivity
- ✓ Add zinc 25mg with dinner, vitamin D3 3000 IU with a fat-containing meal
- ✓ Begin DIM 150mg with your largest meal (if estrogen dominant)
- ✓ Drink 2.5-3L filtered water daily; herbal teas count toward total
- ✓ Eliminate snacking if possible — allow insulin to drop between meals
- ✓ Journal: energy levels 1-10 at waking, midday, 3pm, bedtime
- ✓ Begin morning adrenal cocktail: juice of $\frac{1}{2}$ orange, $\frac{1}{4}$ tsp sea salt, $\frac{1}{4}$ tsp cream of tartar

WEEK 3: LIVER & GUT ACTIVATION

- ✓ Add milk thistle silymarin 150mg 3x daily with meals
- ✓ Start fermented food daily: 2 tbsp kimchi OR ½ cup kefir OR ¼ cup sauerkraut
- ✓ Add calcium D-glucarate 500mg with meals (2-3x daily if estrogen dominant)
- ✓ Begin high-quality probiotic 50 billion CFU with dinner
- ✓ Add dandelion root tea daily (liver and bile support)
- ✓ Take NAC 600mg away from food (morning or afternoon) for glutathione
- ✓ Ensure daily bowel movements — if constipated, add magnesium citrate 200mg and increase fiber
- ✓ Add 1 tbsp ground flaxseed daily to smoothie, yogurt, or oatmeal

WEEK 4: ADRENAL SUPPORT BEGINS

- ✓ Start ashwagandha KSM-66 300mg morning and 300mg evening
- ✓ Add B-complex (activated, methylated forms) with breakfast
- ✓ Implement 10-minute daily breathwork practice (box breathing or 4-7-8)
- ✓ Create non-negotiable tech boundaries: no email after 8pm, no phone in bedroom
- ✓ Begin cold water face immersion 30 seconds every morning (vagus nerve activation)
- ✓ Weekly magnesium epsom salt bath: 2 cups in hot water, 20-minute soak
- ✓ Review all sources of chronic stress and identify top 3 to address
- ✓ Begin Vitex 400mg every morning on empty stomach (if progesterone type)

WEEK 5: CYCLE SYNCING BEGINS

- ✓ Track your cycle using Clue or MyFLO app — identify your current phase
- ✓ Begin seed cycling: flaxseed + pumpkin seeds in follicular, sesame + sunflower in luteal (1 tbsp each)
- ✓ Adjust exercise to cycle phase (see Section 7 for full guide)

- ✓ Adjust carbohydrate intake to cycle phase: lower follicular, moderate increase luteal
- ✓ Add specific Phase support supplements based on your hormone type
- ✓ Review sleep quality — add L-theanine 200mg or low-dose melatonin 0.5mg if still disrupted
- ✓ Mid-program check: re-rate your 10 symptoms and compare to Week 1 baseline
- ✓ Celebrate what's already improved — acknowledge your progress!

WEEK 6: DEEP OPTIMIZATION

- ✓ DUTCH test or mid-luteal blood test to assess progress (optional but valuable)
- ✓ Add targeted nutritional support based on ongoing symptoms
- ✓ Address any remaining gut issues: consider SIBO protocol if bloating persists
- ✓ Introduce castor oil packs over liver 3x weekly (flannel cloth, castor oil, heat)
- ✓ Add collagen peptides 10-15g daily for gut lining and hormone precursors
- ✓ Review supplement protocol and remove anything not providing benefit
- ✓ Social connection focus: loneliness significantly elevates cortisol; prioritize relationships
- ✓ Try near-infrared light therapy or sauna if available (cortisol and metabolic support)

WEEK 7: CONSOLIDATION

- ✓ Full symptom re-rating — compare carefully to Week 1 baseline
- ✓ Slowly begin reintroducing eliminated foods one at a time, 3 days apart, monitoring reactions
- ✓ Simplify supplement stack to sustainable maintenance essentials
- ✓ Establish your personal hormone maintenance routine (morning/evening ritual)
- ✓ Build sustainable 80/20 principle: aim for 80% compliance, allow 20% flexibility without guilt
- ✓ Schedule 3-month retesting of key hormones
- ✓ Share your wins and what you've learned about your unique body
- ✓ Plan your quarterly check-in and maintenance protocol

WEEK 8: MAINTENANCE & FORWARD PLANNING

- ✓ Final full symptom assessment and celebration of your transformation
- ✓ Document what worked best for YOU specifically — your personal protocol blueprint
- ✓ Set up quarterly hormone monitoring schedule
- ✓ Create a hormone crisis plan for high-stress periods (travel, illness, work crunch)
- ✓ Plan seasonal adjustments: more detox support in spring, more adrenal support in winter
- ✓ Explore additional functional testing if any symptoms remain: DUTCH, GI Map, genetics
- ✓ Connect with a functional medicine practitioner if you want guided next steps
- ✓ You have everything you need — trust your body's wisdom and keep learning

SECTION 7: CYCLE-SYNCD EXERCISE

Why Women Should Not Train Like Men

Men have stable hormone levels day to day. Women's hormones fluctuate dramatically across a 28-day cycle, creating four distinct physiological phases with different energy availability, recovery capacity, strength potential, injury risk, and metabolic demands. Training the same way every day ignores this reality — it's why many women feel great some weeks and exhausted others, and mistake that for laziness or lack of discipline.

Estrogen (dominant in follicular phase) enhances muscle protein synthesis, glycogen storage, endurance, and motivation. Progesterone (dominant in luteal phase) raises body temperature, promotes fatigue, and breaks down muscle tissue slightly. This means your body is literally a different training environment week to week.

Phase	Days	Dominant Hormones	Physical Capacity	Best Exercise	Avoid
Menstrual	1-5	E+P lowest	Lower energy, heightened pain sensitivity, inward focus	Yin yoga, gentle walking, swimming, restorative movement	High intensity, heavy lifting, races
Follicular	6-13	Estrogen rising	Increasing energy and strength, coordination improving, motivated	HIIT, heavy strength training, high cardio, new skills	Nothing — peak capacity !
Ovulatory	14-17	E+T peak, LH surge	Maximum performance window, peak strength and endurance	PR attempts, competitions, maximum intensity, sprints	Be alert for ligament laxity injury risk

Phase	Days	Dominant Hormones	Physical Capacity	Best Exercise	Avoid
Early Luteal	18-22	Progesterone rising	Good but declining; still capable of moderate intensity	Moderate strength (maintenance loads), yoga, pilates, moderate cardio	New PRs, maximum intensity
Late Luteal	23-28	P drops, E fluctuating	Fatigue increasing, PMS possible, restorative needed	Walking, gentle yoga, light weights, stretching	Intense exercise can worsen PMS significantly

SECTION 8: ENVIRONMENTAL TOXINS & XENOESTROGENS

What Are Xenoestrogens?

Xenoestrogens are synthetic chemicals that mimic estrogen in the body. They bind to estrogen receptors, triggering estrogenic responses even when your own estrogen levels are normal or high. Exposure is cumulative — no single product may seem harmful, but daily exposure to dozens of sources creates a significant estrogenic burden. Women are particularly vulnerable because we use more personal care products on average and because our hormonal systems are more finely calibrated to estrogenic signals.

Toxin	Found In	Health Impact	Safer Alternative
BPA/BPS	Plastic containers, can linings, receipts	Estrogen mimicry, thyroid disruption, fertility impacts	Glass, stainless steel, BPA-free cans (still contains BPS — best is glass)
Phthalates	Fragrance in all products, PVC plastics, nail polish	Androgen disruption, reduced testosterone, fertility impacts	Fragrance-free products; check EWG Skin Deep database
Parabens	Cosmetics, shampoos, conditioners, skincare	Weak estrogen mimicry, detected in breast tumor tissue	'Paraben-free' labeled products; make your own with oils
Pesticide residues	Conventional produce (especially Dirty Dozen)	Multiple hormone disruption mechanisms, endocrine interference	Organic for Dirty Dozen; wash conventional produce with baking soda

Toxin	Found In	Health Impact	Safer Alternative
PFAS	Non-stick cookware, waterproof clothing, food packaging	Thyroid disruption, immune impacts, persistent in body	Cast iron, stainless, or ceramic cookware; avoid fast food packaging
Triclosan	Antibacterial soaps and personal care products	Thyroid disruption, antibiotic resistance	Regular soap — just as effective, no downsides
Tap water contaminants	Atrazine, fluoride, chlorine, pharmaceuticals	Multiple endocrine disruption pathways	Berkey or reverse osmosis filter; avoid fluoride where possible

SECTION 9: SLEEP & HORMONE REGENERATION

Why Sleep Is the Most Powerful Hormone Intervention

Sleep is not passive recovery — it is the most anabolic, hormone-regenerative time of your day. During deep sleep stages 3 and 4, the pituitary releases up to 70% of its daily growth hormone output, which repairs tissues, builds muscle, regulates insulin sensitivity, and supports metabolic health. The cortisol diurnal rhythm depends entirely on sleep quality: good sleep creates the proper morning cortisol peak (which is essential for energy and focus) and the proper evening cortisol drop (which is essential for falling asleep). Disrupt sleep, and you disrupt cortisol — which then disrupts every other hormone.

A single night of under 6 hours of sleep measurably increases cortisol by 30%, reduces insulin sensitivity to prediabetic levels, drops testosterone by 10-15%, impairs progesterone production, and elevates inflammatory markers that dysregulate estrogen signalling. These are not trivial effects — they are clinically significant changes happening in a single night. Chronic sleep deprivation is one of the most powerful hormone disruptors you can experience.

Sleep Issue	Hormone Disrupted	Mechanism	Solution
Can't fall asleep	Cortisol, melatonin	Cortisol still elevated at bedtime due to evening light/stress	Blue light blocking glasses, no screens 90 min before bed, L-theanine 200mg
Wake 2-4am	Blood sugar, cortisol	Blood sugar drops trigger cortisol release to raise it	Small protein snack before bed (1 tbsp almond butter); review blood sugar
Night sweats	Estrogen, cortisol	Estrogen fluctuations (perimenopause) or high cortisol causing vasodilation	Magnesium 400mg evening, cool bedroom 65-68°F, estrogen support protocol

Sleep Issue	Hormone Disrupted	Mechanism	Solution
Non-restorative sleep	Thyroid, cortisol	Low thyroid reduces deep sleep time; high cortisol prevents deep stages	Thyroid workup; cortisol pattern testing; magnesium glycinate
Vivid dreams	Estrogen, B6	Hormonal fluctuations affect neurotransmitters during REM	Cycle phase awareness — normal in follicular; B6 if excessive

The Complete Sleep Optimization Protocol

- Keep bedroom at 65-68°F (18-20°C) — body temperature must drop to initiate and maintain deep sleep
- Blackout curtains: even small amounts of light through closed eyelids suppress melatonin
- No screens 60-90 minutes before bed OR use blue-light blocking glasses from sunset
- Consistent bedtime $\pm$ 30 minutes every night, including weekends — critical for circadian rhythm
- Morning bright light exposure within 30 min of waking: anchors circadian clock for better sleep that night
- Magnesium glycinate 300-400mg 1 hour before bed: relaxes muscles, lowers cortisol, promotes GABA
- Avoid alcohol: it dramatically suppresses REM sleep and melatonin, causing fragmented sleep architecture
- Low-dose melatonin 0.5mg (not 10mg): signals circadian rhythm; high doses suppress natural production
- L-theanine 200-400mg: promotes relaxed-alert state, reduces cortisol without sedation
- Apigenin 50mg (from chamomile): binds GABA receptors; promotes sleep onset without grogginess

SECTION 10: SUPPLEMENT PROTOCOL DEEP DIVE

How to Build Your Hormone Reset Stack

Supplements are powerful tools, but only after the foundation is in place. Nutrition, sleep, stress management, and toxin reduction come first — supplements amplify a good foundation but cannot replace it. Start with the foundation stack (magnesium, omega-3, vitamin D) in week 1, then add targeted supplements based on your hormone type in weeks 3-4, and advanced support in weeks 5-6.

Week	Add These Supplements	Why Now
1-2	Magnesium glycinate 300mg, Omega-3 2g, Vitamin D3 3000 IU + K2	Foundation — addresses the most universal deficiencies; safe and broadly supportive
3-4	DIM 150mg (if estrogen dominant), Calcium D-glucarate 500mg, NAC 600mg, B-complex	Liver and detox activation; timing matters — start after food changes are in place
4-5	Ashwagandha KSM-66 300mg 2x, Vitex 400mg (if progesterone deficient), B6 P5P 50mg	HPA axis and sex hormone support; adrenal adaptogens need baseline stabilization first
5-6	Zinc 25mg, Selenium 200mcg, Inositol 4g (if PCOS), Berberine 500mg (if insulin resistant)	Targeted mineral and metabolic support; add based on your specific hormone type
Ongoing	Review and simplify to 5-6 core supplements; retest to confirm what's still needed	Sustainability matters more than comprehensiveness — a few taken consistently beat many taken sporadically

SECTION 11: TRACKING PROGRESS

Symptom Tracking Matrix

Symptom Area	Week 1 (Baseline)	Week 2	Week 4	Week 6	Week 8
Energy 1-10					
Sleep quality 1-10					
Mood stability 1-10					
Period pain 1-10					
Bloating 1-10					
Brain fog 1-10					
Sex drive 1-10					
PMS severity 1-10					
Skin clarity 1-10					
Hair shedding 1-10					
Stress resilience 1-10					
Cycle regularity					

SECTION 12: TROUBLESHOOTING & CASE STUDIES

Common Challenges & Solutions

Feeling worse at weeks 2-3

This is a healing reaction — completely normal and temporary. Your liver is mobilizing stored estrogen and toxins. Increase water to 3L, add more fiber, take extra magnesium, and reduce detox supplements temporarily. It typically resolves within 7-10 days.

Weight hasn't shifted after 4 weeks

Hormonal weight is notoriously slow to respond. First check: are you eating enough? Under-eating is the #1 reason women stall — cortisol spikes and shuts down fat burning. Also check sleep quality and stress levels.

Period became more irregular

Temporary disruption is common as cycles recalibrate. If it persists beyond 3 full cycles or becomes more than 45 days or fewer than 21 days, see a practitioner. Vitex can initially alter cycle length before normalizing it.

Acne got worse

This is often a detox reaction as the liver mobilizes stored androgens and estrogen. Ensure daily bowel movements (critical), reduce sugar completely, add zinc 30mg and liver support. Usually resolves by week 6.

Still exhausted after 6 weeks

Test ferritin (iron stores) — optimal is 70-100, not just 'normal.' Also test B12 (optimal >600), and full thyroid panel. Also honestly assess sleep duration and quality — 7-9 hours in a dark, cool room is non-negotiable.

Case Study: Sarah, 34 — Estrogen Dominance

Sarah presented with increasingly heavy, clotted periods over 3 years, severe PMS for 10+ days monthly, debilitating breast tenderness, unexplained weight gain of 12 lbs despite careful diet, and a complete loss of libido. Mood swings in the week before her period were affecting her marriage. Testing revealed estradiol 487 pg/mL (optimal <400 mid-cycle), progesterone 4.2 ng/mL (should be 15-25 in luteal phase), elevated beta-glucuronidase on gut testing, and sluggish Phase 2 liver methylation.

Her 12-week protocol: DIM 200mg + calcium D-glucarate 1g daily; Vitex 600mg every morning; comprehensive gut repair (probiotic, prebiotic, L-glutamine, digestive enzymes); cruciferous vegetables at every meal; complete alcohol elimination; castor oil liver packs 3x weekly; NAC 600mg. The results at 3 months: period pain reduced 80%, PMS reduced from 10 days to 3 manageable days, breast tenderness resolved completely, 8 lbs released without calorie restriction. Retesting: estradiol 318 pg/mL, progesterone 19 ng/mL.

Case Study: Maria, 43 — Perimenopause + Cortisol Overload

Maria was experiencing 8-10 hot flashes daily, night sweats disrupting sleep, cycles varying 21-48 days, memory lapses she feared were dementia, and anxiety that seemed to come from nowhere. She was running a business and working 60-hour weeks. Her DUTCH test showed a flat cortisol curve (Stage 2 HPA dysregulation), barely detectable progesterone, wildly fluctuating estrogen, and DHEA-S at the bottom of the range.

The critical insight was that her adrenals needed to be addressed before anything else — no amount of hormone support would work in the context of HPA dysfunction. Protocol: work reduction to 45 hours (non-negotiable), ashwagandha and rhodiola, complete dietary overhaul to stabilize blood sugar, sleep protocol. After adrenal stabilization at 6 weeks, added Vitex, then bioidentical progesterone cream with practitioner guidance. At 16 weeks: hot flashes reduced to 1-2 per day, sleep normalizing, memory clearing, cycles 26-34 days.

FREQUENTLY ASKED QUESTIONS

Q: How long until I see real results?

A: Most women notice energy and sleep improvement within 2-4 weeks. Cycle changes require 2-3 full cycles (60-90 days). Structural changes (fibroid size, cyst resolution) take 6-12 months. Be patient — the

root causes took years to develop.

Q: Can I follow this while on hormonal birth control?

A: Yes, but note the pill artificially controls hormone levels and depletes B6, B12, zinc, magnesium, and folate significantly. Focus on replenishing these nutrients. The gut health, liver support, and lifestyle components all still apply.

Q: What's the minimum I must do if I can't do everything?

A: Non-negotiables: sleep 7-9 hours, eat protein + fat at every meal, eliminate alcohol and refined sugar, take magnesium glycinate nightly, manage stress. Everything else builds on this foundation.

Q: Is it normal to feel worse before better?

A: Yes — a healing reaction (Herxheimer-like response) in weeks 2-3 is common as the liver mobilizes stored hormones and toxins. Increase water, fiber, and rest. It typically resolves within 7-10 days.

Q: Should I test my hormones before starting?

A: Ideal but not required. If you can, get a mid-luteal blood panel (day 19-22) including E2, progesterone, testosterone, DHEA-S, and full thyroid. This lets you track objective improvement at 3 months.

RESOURCES & REFERENCES

Recommended Reading: 'The Hormone Cure' — Dr. Sara Gottfried | 'Period Repair Manual' — Lara Briden ND | 'Woman Code' — Alisa Vitti | 'Estrogen Matters' — Dr. Avrum Bluming & Carol Tavis | 'The Menopause Brain' — Dr. Lisa Mosconi

Lab Testing: DUTCH Complete (dutchtest.com) | ZRT Laboratory (zrtlab.com) | Vibrant Wellness | LabCorp and Quest Diagnostics (order through your practitioner or direct-to-consumer services)

Find a Practitioner: Institute for Functional Medicine (ifm.org) | The DUTCH Test practitioner finder | SteadyMD functional medicine telemedicine | NAMS (North American Menopause Society) provider

locator

Flora Wellness Community: Join our private community at florawellness.com/community for ongoing support, weekly Q&As, and accountability with thousands of women on the same journey.